

IBN SINA EMERGENCY DEPARTMENT

Paediatric Encounter Record - Clinical Data Sheet

SYNTHETIC RECORD - FICTIONAL PATIENT - NOT REAL PATIENT DATA
Generated for research prototype evaluation. Not for clinical use.

1. PATIENT IDENTIFICATION AND DEMOGRAPHICS

Medical record number	IBN-001
Patient name	Yousef Mahmoud Ali
Date of birth	2022-03-11
Age at presentation	4 years
Sex	Male
Accompanying guardian	Mother - Hoda Ali (present, primary historian)
ED arrival date/time	2026-08-13 21:42
Mode of arrival	Ambulatory, brought by family

2. INSURANCE AND ADMINISTRATIVE

Provider	National Health Insurance Authority (NHIA)
Policy number	NHIA-4471-88213
Plan	Family - Category B
Coverage status	Active, verified at registration
Co-payment	EGP 50 ED attendance; imaging covered
Referral required	No (emergency presentation)

3. PRESENTING COMPLAINT

Chief complaint Fever and difficulty breathing for 4 days, worsening today

History of presenting illness

Four days of fever up to 39.5 C, initially with dry cough that has become productive. Progressive tachypnoea over the last 48 hours. Today the mother noted grunting, chest indrawing and reluctance to feed. Reduced activity and increasing drowsiness since this morning. Two episodes of vomiting after coughing. No diarrhoea, no rash, no seizure. No recent travel. Attends nursery; two classmates recently unwell with fever and cough.

4. TRIAGE OBSERVATIONS AND VITAL SIGNS

Parameter	Value	Interpretation
Temperature (axillary)	39.4 C	HIGH
Respiratory rate	52 /min	HIGH
Heart rate	148 /min	HIGH
SpO2 (room air)	88 %	CRITICAL
Blood pressure	92/58 mmHg	Normal
Capillary refill	3 seconds	PROLONGED
Weight	16.2 kg	50th centile
AVPU	V - responds to voice	REDUCED

5. CLINICAL EXAMINATION FINDINGS

- Marked subcostal and intercostal recession; audible grunting at rest.
- Nasal flaring present. Head bobbing noted intermittently.
- Reduced air entry right lower zone with bronchial breathing and coarse crackles.
- Dullness to percussion right base.
- No stridor. No wheeze. No rash. Neck supple, no meningism.
- Refusing all oral fluids for the past 12 hours. Dry mucous membranes.
- Last wet nappy approximately 9 hours ago.

6. PAST MEDICAL, DRUG, FAMILY AND SOCIAL HISTORY

Birth history	Term (39+2), spontaneous vaginal delivery, birth weight 3.2 kg. No NICU admission.
Developmental	Milestones age-appropriate. Attends nursery. Speech and gait normal.
Immunisations	Up to date per national schedule EXCEPT PCV13 booster (missed, due age 12 months). BCG, OPV/IPV, pentavalent, MMR all documented.
Past medical history	Two prior episodes of wheeze with viral URTI (ages 2 and 3), no maintenance inhaler. Otherwise well.
Past surgical history	None.
Prior hospital admissions	None.
Current medications	Paracetamol 15 mg/kg PRN (last dose 4 hours ago). Ibuprofen given by mother yesterday.
Allergies	No known drug allergies.
Family history	Father has asthma. Maternal grandfather - type 2 diabetes. No TB contact known.
Social history	Lives with both parents and two siblings, Cairo. Household smoker (father, smokes indoors). Biomass cooking fuel not used.

7. LABORATORY RESULTS

Results available at the time of clinician review.

Investigation	Result	Reference range	Flag
White cell count	21.3 x10 ⁹ /L	4.0-12.0	HIGH
Neutrophils	82 %	30-60	HIGH
Lymphocytes	12 %	30-60	LOW
Haemoglobin	10.2 g/dL	11.5-14.5	LOW
Platelets	418 x10 ⁹ /L	150-450	Normal
C-reactive protein	168 mg/L	<5	HIGH
Procalcitonin	6.2 ng/mL	<0.5	HIGH
Sodium	132 mmol/L	135-145	LOW
Potassium	4.1 mmol/L	3.5-5.1	Normal
Urea	7.8 mmol/L	2.5-6.5	HIGH
Creatinine	38 umol/L	20-50	Normal
Venous lactate	2.8 mmol/L	<2.0	HIGH
Blood glucose	6.4 mmol/L	3.5-7.0	Normal
Blood culture	Collected pre-antibiotic; result pending	-	PENDING
Respiratory viral PCR	Negative for influenza A/B, RSV, SARS-CoV-2	Negative	Normal

8. IMAGING REQUEST AND LINKED STUDY

Imaging order	Chest radiograph, AP projection, STAT. Indication: suspected lower respiratory tract infection with hypoxaemia.
Linked image file	chest_xray/test/PNEUMONIA/person3_bacteria_10.jpeg
Image source	Kermany et al. chest X-ray dataset (CC BY 4.0)
Radiographer notes	Child unable to stand for PA film; AP supine obtained. Inspiration adequate. Mild rotation to the right.
Radiology report	NOT YET REPORTED - awaiting interpretation

9. RECORD STATUS

Diagnosis	NOT YET ESTABLISHED - pending clinician assessment
Disposition	NOT YET DECIDED - pending clinician assessment
Record completeness	Registration, triage, clerking, labs and imaging request complete
Data classification	SYNTHETIC - fictional patient, safe for third-party API processing